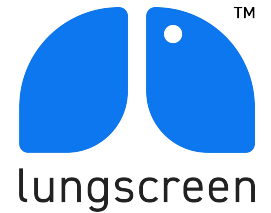


CHEST RADIOGRAPH CLASSIFICATION

Adjudicated ILO B-read Report
Lungscreen Australia Pty Ltd
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Patient
First Name

Last Name

Date of birth

Date of Radiograph

Lungscreen
ID Number

Note: Please record your interpretation of a single radiograph by placing an "x" in the appropriate boxes on this form. Classify all appearances described in the ILO International Classification of Radiographs of Pneumoconiosis or Illustrated by the ILO Standard Radiographs. Use symbols and record comments as appropriate.

1. IMAGE QUALITY		Overexposed (dark)	Improper position	Underinflation	Other (please specify)
1	2 3 U/R	Underexposed (light)	Poor contrast	Mottle	
(If not Grade 1, mark all boxes that apply)		Artifacts	Poor processing	Excessive Edge Enhancement	
2A. ANY CLASSIFIABLE PARENCHYMAL ABNORMALITIES?				YES	Complete Sections 2B and 2C NO Proceed to Section 3A
2B. SMALL OPACITIES		b. ZONES	c. PROFUSION	2C. LARGE OPACITIES	
a. SHAPE/SIZE PRIMARY SECONDARY		R L	0/- 0/0 0/1	SIZE O A B C Proceed to Section 3A	
p s	p s	UPPER	1/0 1/1 1/2		
q t	q t	MIDDLE	2/1 2/2 2/3		
r u	r u	LOWER	3/2 3/3 3/+		
3A. ANY CLASSIFIABLE PLEURAL ABNORMALITIES?				YES	Complete Sections 3B, 3C NO Proceed to Section 4A
3B. PLEURAL PLAQUES		(mark site, calcification, extent, and width)		Width (in profile only) (3mm minimum width required)	
Chest wall	Site	Calcification	Extent (chest wall; combined for in profile and face on)		
In profile	O R L	O R L	Up to 1/4 of lateral chest wall = 1	3 to 5 mm = a	
Face on	O R L	O R L	1/4 to 1/2 of lateral chest wall = 2	5 to 10 mm = b	
Diaphragm	O R L	O R L	> 1/2 of lateral chest wall = 3	> 10 mm = c	
Other site(s)	O R L	O R L	1 2 3 1 2 3	O R O L a b c a b c	
3C. COSTOPHRENIC ANGLE OBLITERATION				R L	Proceed to Section 3D NO Proceed to Section 4A
3D. DIFFUSE PLEURAL THICKENING		(mark site, calcification, extent, and width)		Width (in profile only) (3mm minimum width required)	
Chest wall	Site	Calcification	Extent (chest wall; combined for in profile and face on)		
In profile	O R L	O R L	Up to 1/4 of lateral chest wall = 1	3 to 5 mm = a	
Face on	O R L	O R L	1/4 to 1/2 of lateral chest wall = 2	5 to 10 mm = b	
			> 1/2 of lateral chest wall = 3	> 10 mm = c	
			1 2 3 1 2 3	O R O L a b c a b c	
4A. ANY OTHER ABNORMALITIES?				YES	Complete Sections 4B, 4C, 4D, 4E NO Complete physician info and sign form.
4B. OTHER SYMBOLS (OBLIGATORY)					
aa at ax bu ca eg cn co cp cv di ef em es fr hi ho id ih kl me pa pb pi px ra rp tb					
(If other diseases or significant abnormalities (OD), findings must be recorded on reverse. (section 4C/4D) See reverse for other symbol definitions.)					
4E. Should worker see personal physician because of findings?				YES	NO Date Doctor Notified

4B. OTHER SYMBOLS Definitions

Each of the following definition of symbols assumes an introductory qualifying word of phrase such as "changes indicative of" or "opacities suggestive of" or "suspect."

aa	atherosclerotic aorta	hi	enlargement of non-calcified hilar or mediastinal lymph nodes
at	significant apical pleural thickening	ho	honeycomb lung
ax	coalescence of small opacities - with margins of the small opacities remaining visible, whereas a large opacity demonstrates a homogeneous opaque appearance - may be recorded either in the presence or in the absence of large opacities	id	ill-defined diaphragm border - should be recorded only if more than one-third of one hemidiaphragm is affected
bu	bullae(e)	ih	ill-defined heart border - should be recorded only if the length of the heart border affected, whether on the right or on the left side, is more than one-third of the length of the left heart border
ca	cancer, thoracic malignancies excluding mesothelioma	kl	septal (Kerley) lines
cg	calcified non-pneumoconiotic nodules (e.g. granuloma) or nodes	me	mesothelioma
cn	calcification in small pneumoconiotic opacities	pa	plate atelectasis
co	abnormality of cardiac size or shape	pb	parenchymal bands - significant parenchymal fibrotic stands in continuity with the pleura
cp	cor pulmonale	pi	pleural thickening of an interlobar fissure
cv	cavity	px	pneumothorax
di	marked distortion of an intrathoracic structure	ra	rounded atelectasis
ef	pleural effusion	rp	rheumatoid pneumoconiosis
em	emphysema	tb	tuberculosis
es	eggshell calcification of hilar or mediastinal lymph nodes		
fr	fractured rib(s) (acute or healed)		

4C. MARK ALL BOXES THAT APPLY: (Use of this list is intended to reduce handwritten comments and is optional)

Abnormalities of the Diaphragm

Eventration

Hiatal hernia

Airway Disorders

Bronchovascular markings, heavy or increased

Hyperinflation

Bony Abnormalities

Bony chest cage abnormality

Fracture, healed (non-rib)

Fracture, not healed (non-rib)

Scoliosis

Vertebral column abnormality

Lung Parenchymal Abnormalities

Azygos lobe

Density, lung

Infiltrate

Nodule, nodular lesion

Miscellaneous Abnormalities

Foreign body

Post-surgical changes/sternal wire

Cyst

Vascular Disorders

Aorta, anomaly of

Vascular abnormality

4D. OTHER COMMENTS

Patient

First Name:

Last Name:

Patient Date of birth:

Lungscreen ID Number:

5.

Radiology Facility:

Type of reading

Ordering Physician or program:

A B Other

Clinical Director

Date of Reading

Electronic Signature

Classification purpose:

